BASELINE → TREATMENT → FOLLOW-UP

Safety tests before, between and after capecitabine and temozolomide (CAPTEM) cycles

Six cycles are planned in National Cancer Control Programme (NCCP); the team may change or stop treatment for progression or toxicity. Do not substitute a fixed scan/marker timetable from another neuroendocrine tumour (NET) treatment.

Check	Before starting	Treatment / reviews	Between visits	Response review	After a pause or stop
Blood count / kidneys / liver	full blood count (FBC), renal and liver profile	Before each cycle; liver tests also after each cycle	Earlier assessment for infection, bleeding, dehydration or symptoms	Review recovery before another cycle	Liver effects may arise weeks after the final dose
dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) / hepatitis B / international normalised ratio (blood-clotting test) (INR)	DPD before capecitabine unless already tested; hepatitis B surface antigen and core antibody	INR if indicated, especially with warfarin	Watch for unusually severe early toxicity	Specialist review of any DPD-related risk or interacting medicine	Follow specialist and anticoagulation plans
Symptoms / oral medicines	Record usual bowel pattern, intake, mouth, skin and activity	Review adherence, worst symptoms and any doses held	Nausea plan on D10–14; symptom calls on all days	Confirm both prescriptions and the next start date	Follow persistent symptoms and return unused medicine
NET response / duration	Confirm pancreatic NET, goal and six-cycle plan	Scans / hormone markers according to the consultant plan	Track symptoms; report change without waiting for imaging	Balance response, toxicity and patient goals	Discuss follow-up or next treatment after the intended course

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00505 v5 · current Irish CAPTEM regimen · EMA Xeloda product information and patient leaflet · EMA Temodal product information and patient leaflet

Tests in your plan

5-hydroxyindoleacetic acid (5-HIAA)

If part of your plan

WHEN

Only if requested for your hormone syndrome; the team sets the dates.

Measures a breakdown product of serotonin to help assess selected neuroendocrine tumours.

Often a timed urine collection. Follow the laboratory's food and medicine instructions.

Chromogranin A

If part of your plan

WHEN

Only when the team considers it informative for your tumour.

A selected tumour-marker result, interpreted with symptoms and scans.

Acid-suppressing medicines and other conditions can alter the result. It is not a routine test before every treatment; do not stop medicines yourself.

Selected pancreatic hormones

If part of your plan

WHEN

Only for the hormone question identified by your specialist.

Examples include gastrin, insulin or glucagon when the diagnosis or symptoms call for them.

The team names the test, preparation and review date; these are not a standard panel for every patient.

THE WHOLE TREATMENT JOURNEY

Capecitabine + temozolomide (CAPTEM): treatment and checks

Locally advanced or metastatic pancreatic neuroendocrine tumour under NCCP 00505a.

Each cycle lasts 28 days: capecitabine twice daily on Days 1–14; add temozolomide once daily on Days 10–14; take neither anticancer medicine on Days 15–28. NCCP plans six cycles unless progression or unacceptable toxicity changes the plan.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Treatment / reviews	Between visits	Response review	After a pause or stop
Blood count / kidneys / liver NCCP 00505	FBC, renal and liver profile	Before each cycle; liver tests also after each cycle	Earlier assessment for infection, bleeding, dehydration or symptoms	Review recovery before another cycle	Liver effects may arise weeks after the final dose
DPD / hepatitis B / INR NCCP 00505	DPD before capecitabine unless already tested; hepatitis B surface antigen and core antibody	INR if indicated, especially with warfarin	Watch for unusually severe early toxicity	Specialist review of any DPD-related risk or interacting medicine	Follow specialist and anticoagulation plans
Symptoms / oral medicines NCCP 00505	Record usual bowel pattern, intake, mouth, skin and activity	Review adherence, worst symptoms and any doses held	Nausea plan on D10–14; symptom calls on all days	Confirm both prescriptions and the next start date	Follow persistent symptoms and return unused medicine
NET response / duration NCCP 00505	Confirm pancreatic NET, goal and six-cycle plan	Scans / hormone markers according to the consultant plan	Track symptoms; report change without waiting for imaging	Balance response, toxicity and patient goals	Discuss follow-up or next treatment after the intended course

What each check is for

1 CAPTEM · six planned 28-day cycles

Different tablet days

CAPTEM combines two oral chemotherapy medicines to help control an advanced pancreatic NET. The team discusses the intended benefit, side effects and alternatives. NCCP identifies this pancreatic NET use as outside the medicines' licences, requiring the local consent process.

Ask / act: Confirm the current prescription, tests and support medicines.

Sources: NCCP 00505

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-05 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.37